

Primary Diagnosis: Metastatic wild-type NSCLC

Date of Diagnosis: 17.01.2020

Molecular Profile:

- **Driver Mutation:** Negative for EGFR, ALK, ROS1, RET, BRAF, MET (NGS panel)
- **PD-L1 Status:** TPS 30%, CPS 32, IC 1+ (SP263 clone)

Treatment History:

Cisplatin (AUC 5) + Pemetrexed (500 mg/m²) + Pembrolizumab (200 mg q3w) beginning on 07.02.2020

Clinical Course: Mr. Whitmore presented with left hip pain and confusion in January 2020. Biopsy confirmed TTF-1 positive adenocarcinoma. He was commenced on triplet chemoimmunotherapy, with partial response at 3 months.

Progression was observed in October 2020. WBRT (30 Gy in 10 fractions) was administered with transient cognitive impairment. 2L treatment was docetaxel + ramucirumab (q3w), with a modest 4-month disease stabilization.

Subsequent PD in the liver prompted 3L nivolumab, discontinued after 2 cycles due to immune-mediated pneumonitis (Grade 3). Managed with systemic steroids and taper.

Comorbidities:

- Severe COPD (FEV1 38%)
- CAD with prior PCI (2015, on dual antiplatelet therapy)
- Osteoporosis (on denosumab q6mo)
- Hyperuricemia

Final Imaging (September 2021):

- Hepatic expansion of metastatic lesions (largest: 4.3 cm segment IV)
- Diffuse osteoblastic transformation of spine

Date of Death: 18.09.2021 **Cause of Death:** Respiratory failure secondary to progressive pulmonary metastases and hepatic dysfunction

Patient Information:

- **Name:** Marcus Alan Whitmore (*02.10.1950)
- **Patient ID:** SYN037

Signed Dr. Heglund 18.09.2021